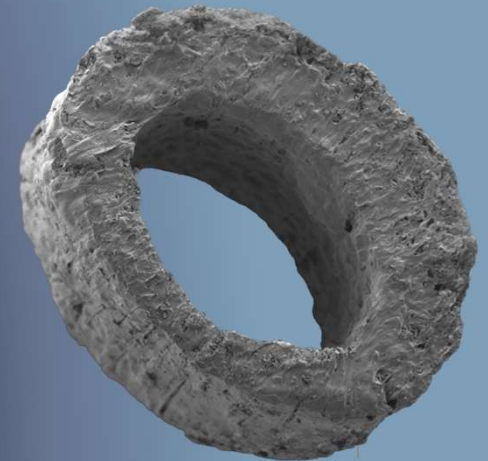


Advances in Interbody Cages for Spinal Fusion

THE SUNDAY TIMES
**T Best Places
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The challenge

Design a cage concept with an **adaptive modulus** to allow the **new bone** that has grown into the pores of the cage to bear an **increasing load over time**.

My top 3 design considerations

1. **Patient Safety**: the ability to bear load until it is certain that new bone will have formed. i.e. aligning temporal adaptive modulus with bone formation - but be cautious.
2. Include design features that **drive bone formation**
3. Ability to **assess** bone formation (radiolucency)



Assessing bone formation

3. Ability to assess bone formation (radiolucency)

An example of the issues it can cause when radiopaque

Research Article

ISSN: 2574-1241



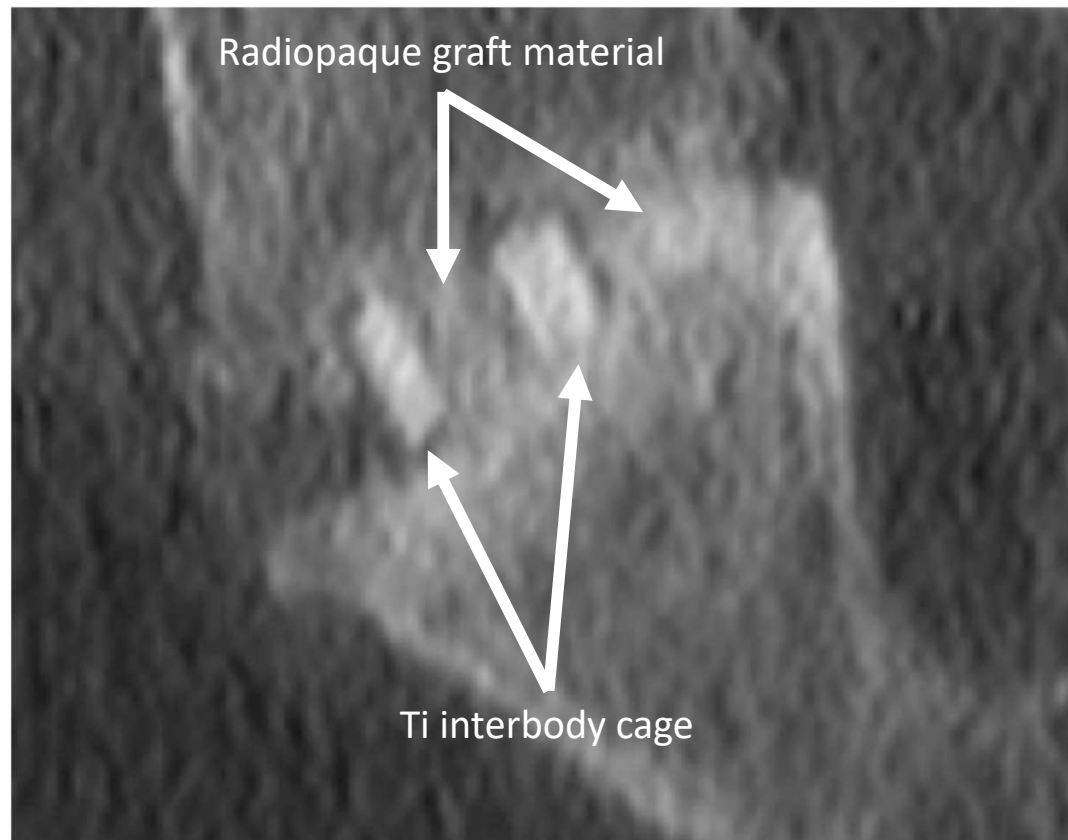
DOI: 10.26717/BJSTR.2024.54.008570

First-In-Human Study with a Novel Synthetic Bone Graft, OssDsign Catalyst™, in Transforaminal Lumbar Interbody Fusion with Instrumented Posterolateral Fusion

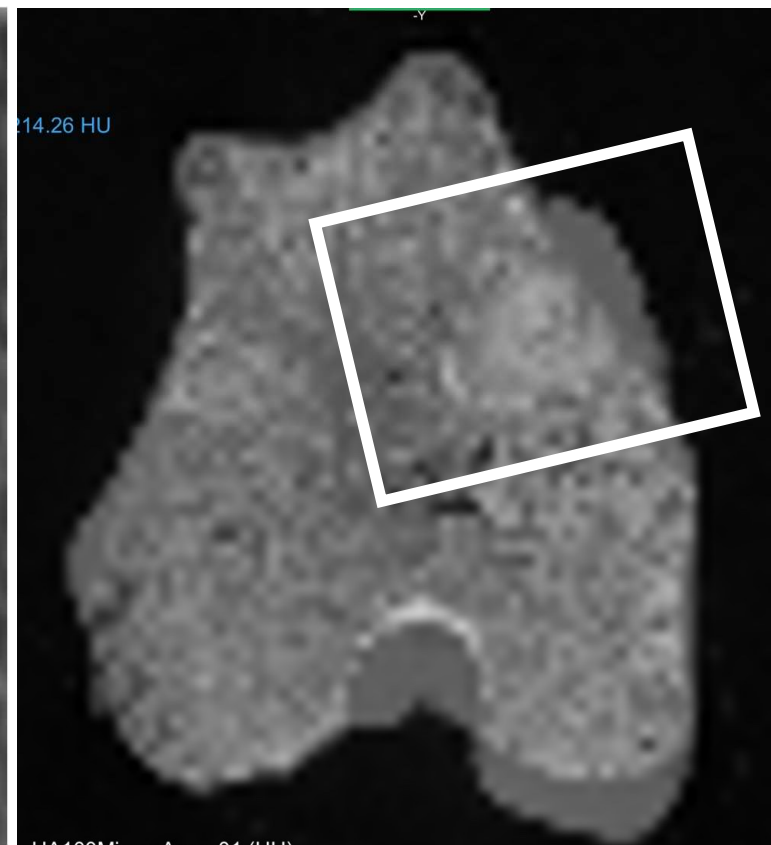
- Reports a 93% fusion rate, which is very high.
- Ceramic (Si-HA) only material (osteoconductive)

Human vs Sheep Defect Study (8x15mm)

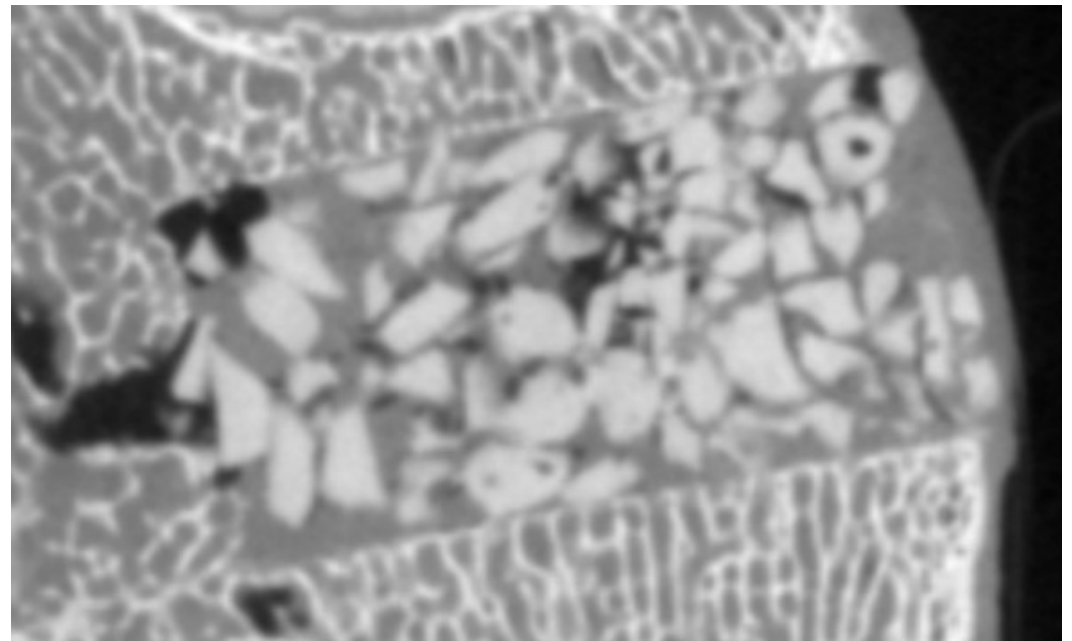
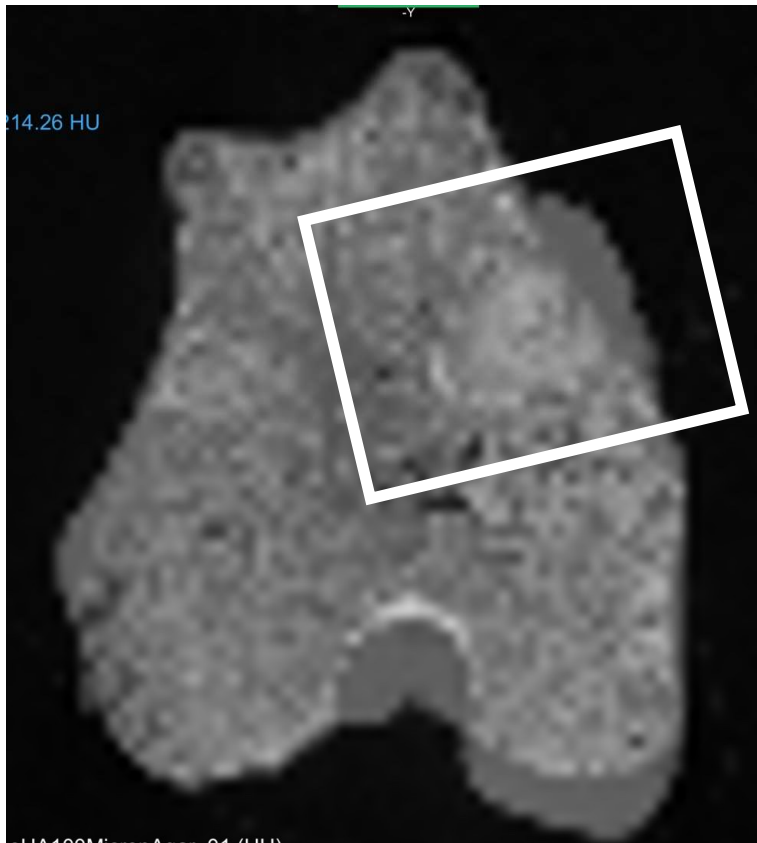
Data from their human clinical study



Data from our preclinical study



A very challenging model (sheep cadaver)



Going further

- ✓ All groups recognised the importance of radiolucency.
- ✓ Some prioritised all polymer-based materials over Ti or other radiopaque materials – but polymers have some disadvantages (see later section on patient safety).
- ✓ Adaptive radiolucent: For example - Group 20's design would be very radiopaque initially, but the resorbing elements would allow increase visualisation of new bone formation over time.





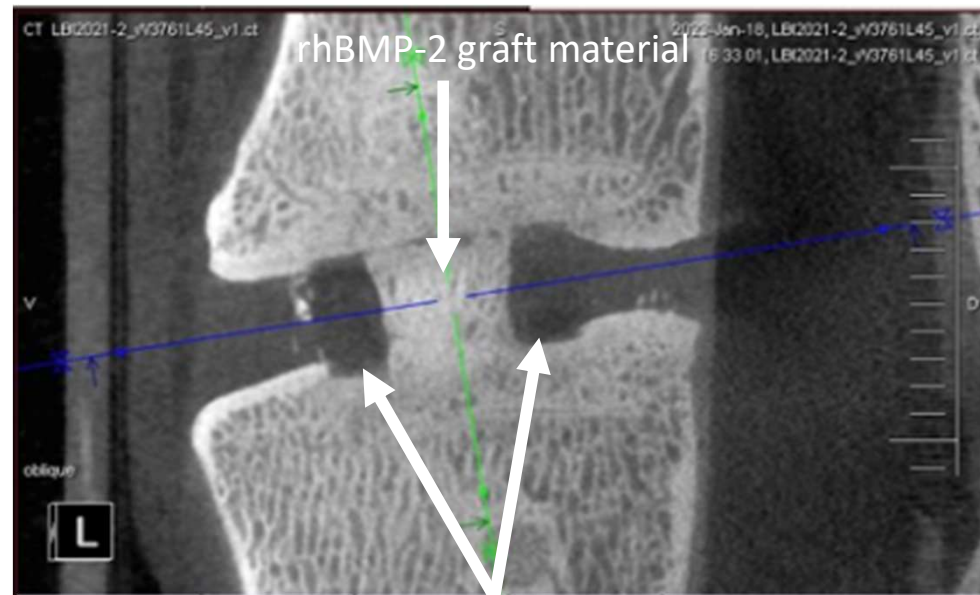
Contributing to bone formation

2. Contribution to bone formation

Surface features considered by the groups

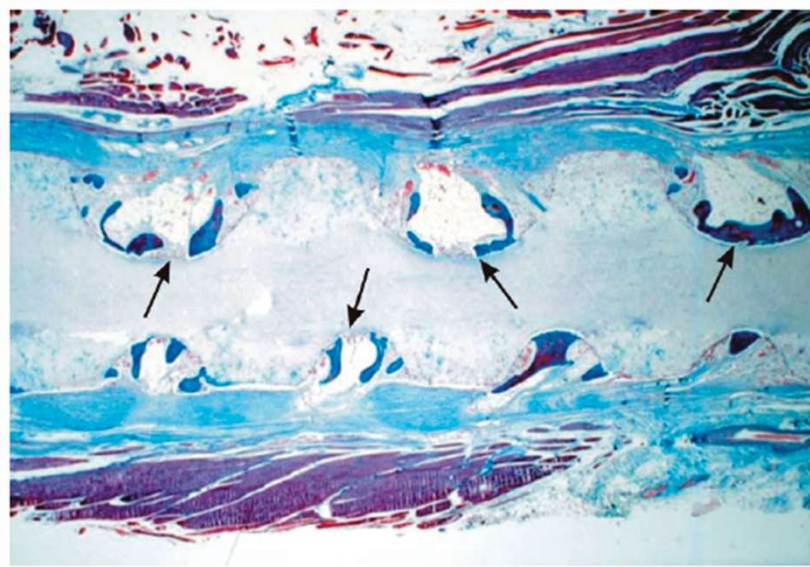
- ✓ Pores (focus on size – waste/nutrients/blood vessels/bone)
- ✓ Bone on-growth e.g. Ti vs PEEK (addressed with HA coating)
- rhBMP-2 coating

An alternative: The use of nanoscale Ti as a coating for PEEK

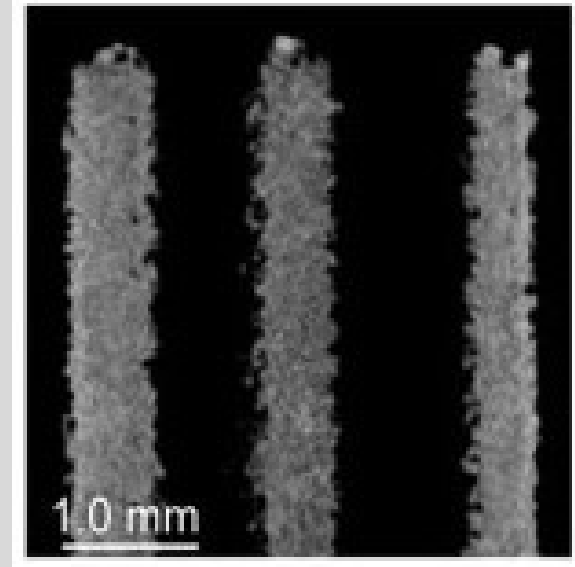


Going 1 step further – shapes and surface

Macro Scale Matters



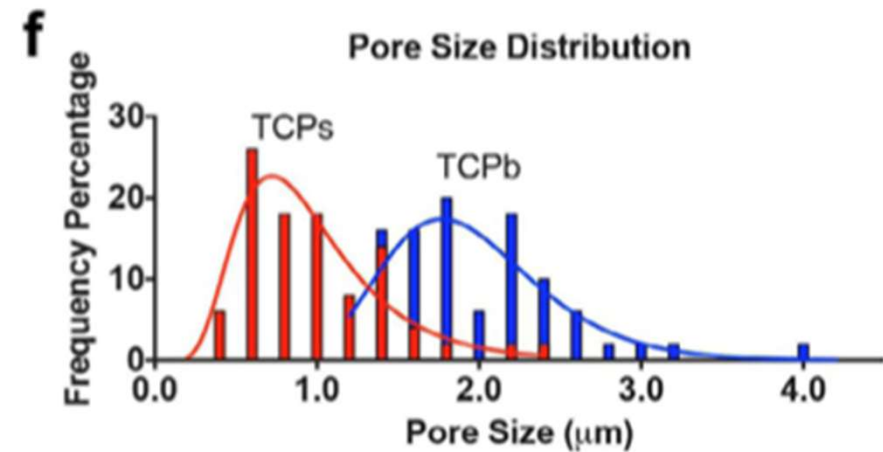
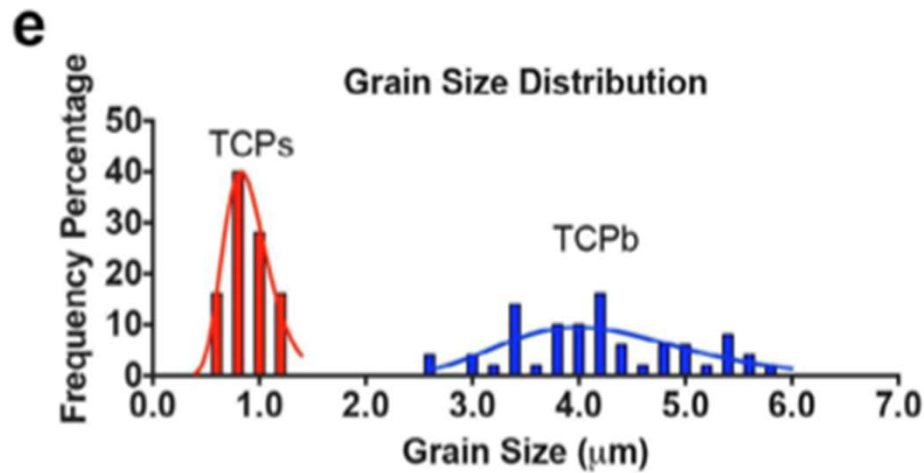
Ripamonti U., J. Cell. Mol. Med. Vol 8, No 2, 2004 pp. 169-180



A. Fukuda et al., Acta Biomater. 7 (5) (2011) 2327

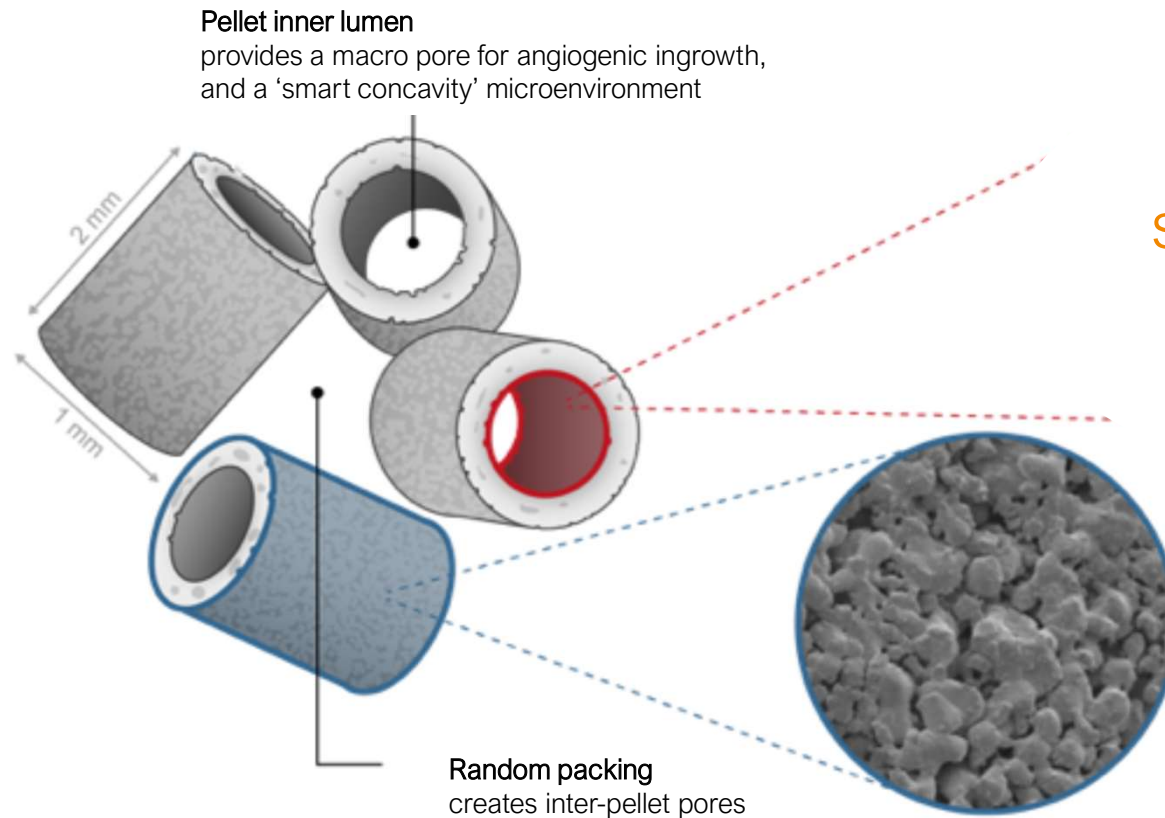
Insight driven design

Nano-scale matters



Insight driven design

In Vitro Testing

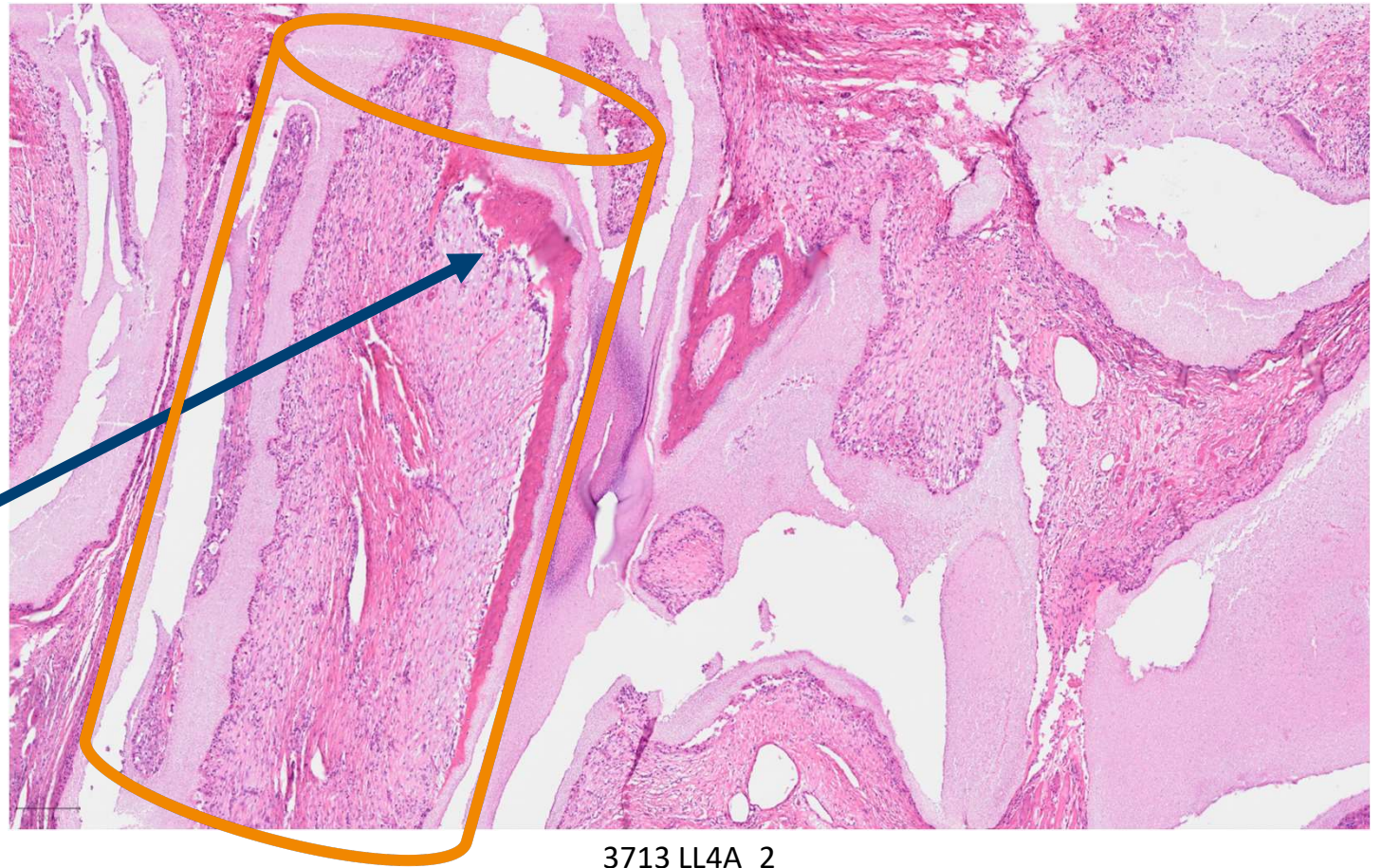


Combines concave
surfaces (tubes) and
submicron surface
features

Testing macro features only (shape)

Intramuscular implantation – no osteoconduction, any new bone would be osteoinduction

New bone at 6 weeks



Testing macro features only (shape)

12 weeks

- Significant new bone at 12 weeks

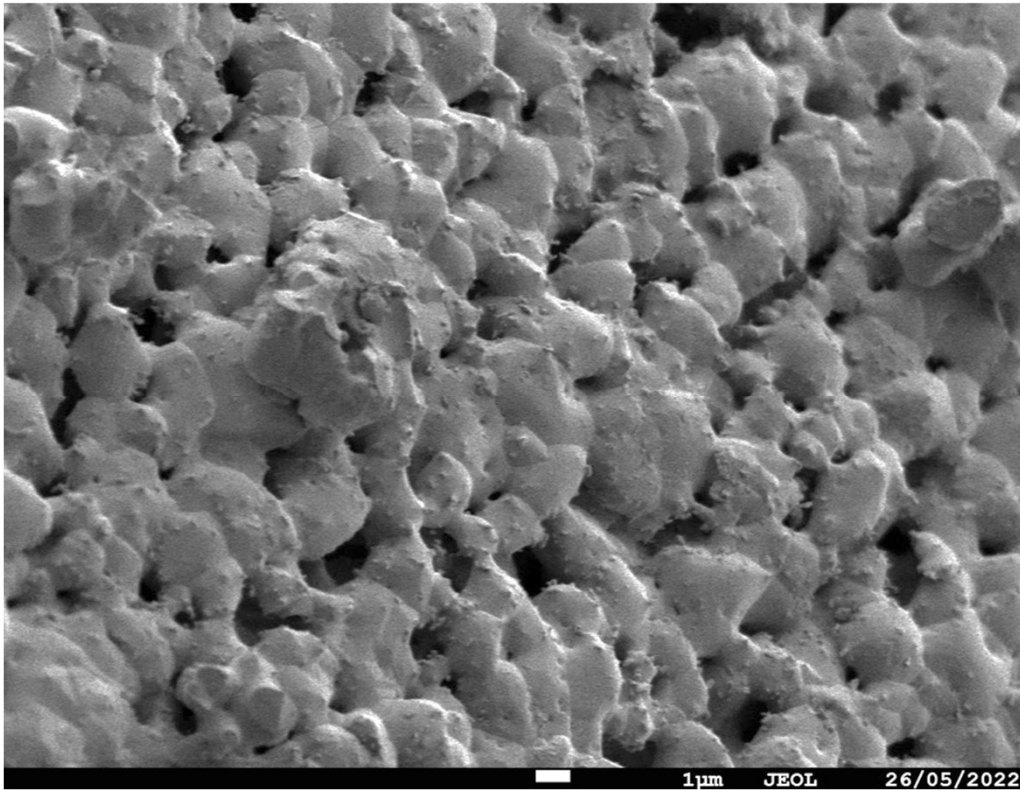
Osteoinduction from a ceramic based just on a concave shape!



SEM – different surface topographies

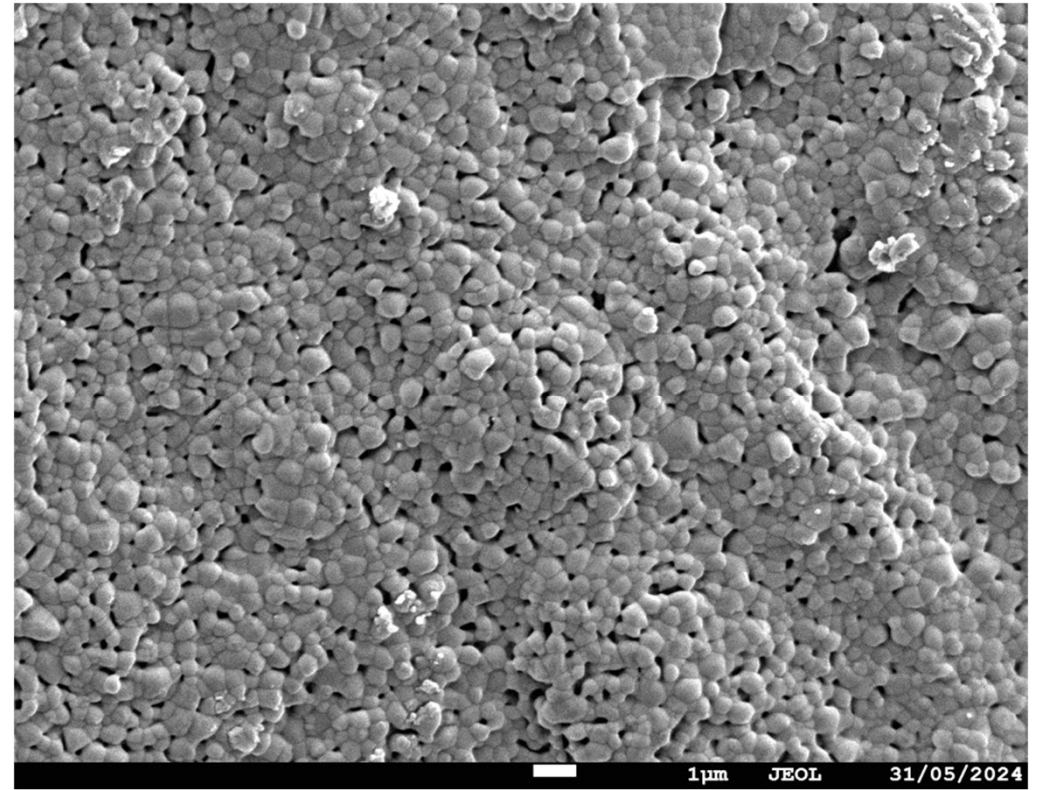
x4000

Non-submicron features



x5000

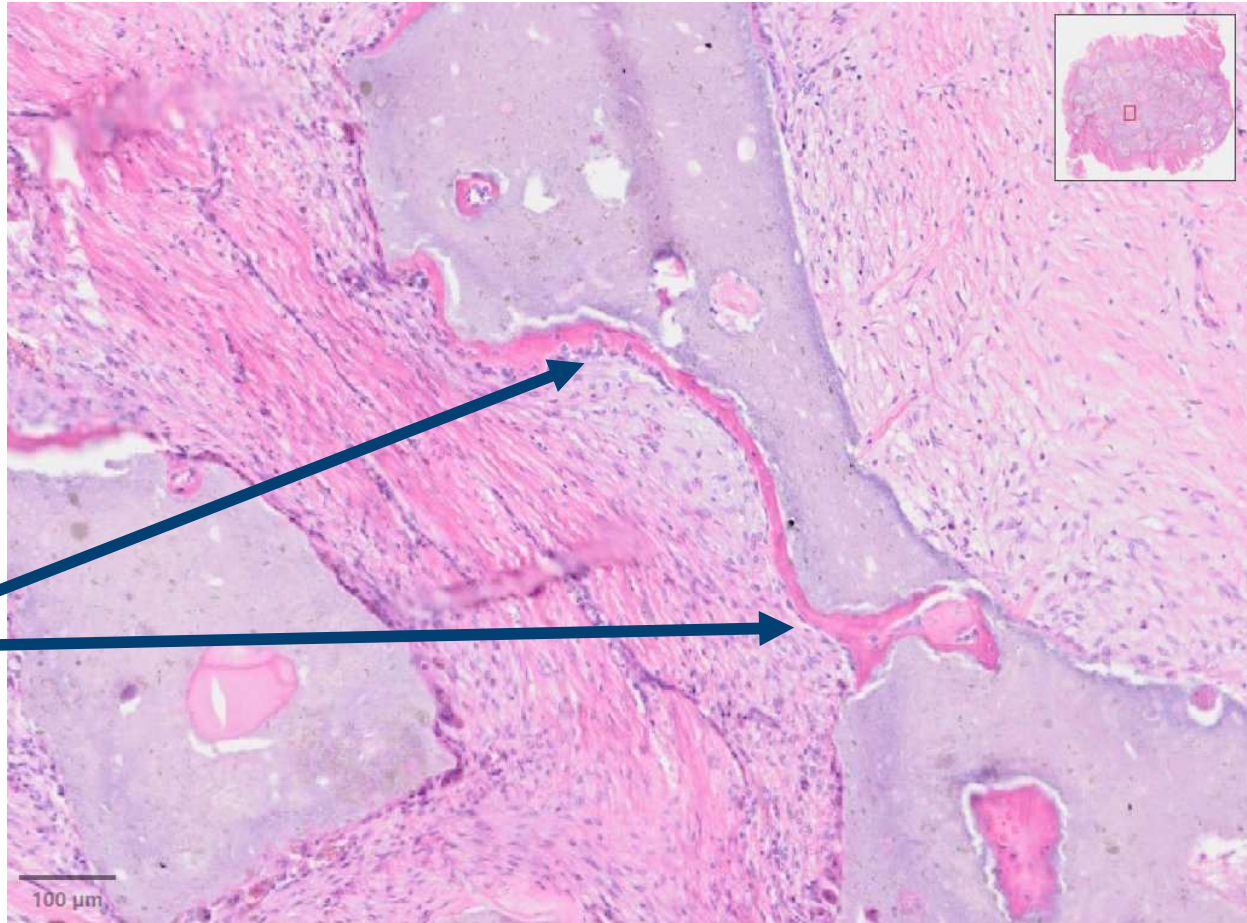
Submicron features



Testing nano-scale features only

Osteoinduction from a ceramic based just on a nano-scale surface features!

New bone at 6 weeks



Going further with your designs

Think about

- **Nanoscale features** on the surface of your cages (easier to do with Ti)
- Not just size of your pores, but also their **shapes** (concave) – lots of options with 3D printing
- Add a permanent **surface charge** – acts to attract and bind endogenous growth factors to the surface
- Consider **Piezoelectric** materials (e.g. polyvinylidene fluoride – PVDF)

Patient Safety

1. Patient Safety

Polymer only cages....

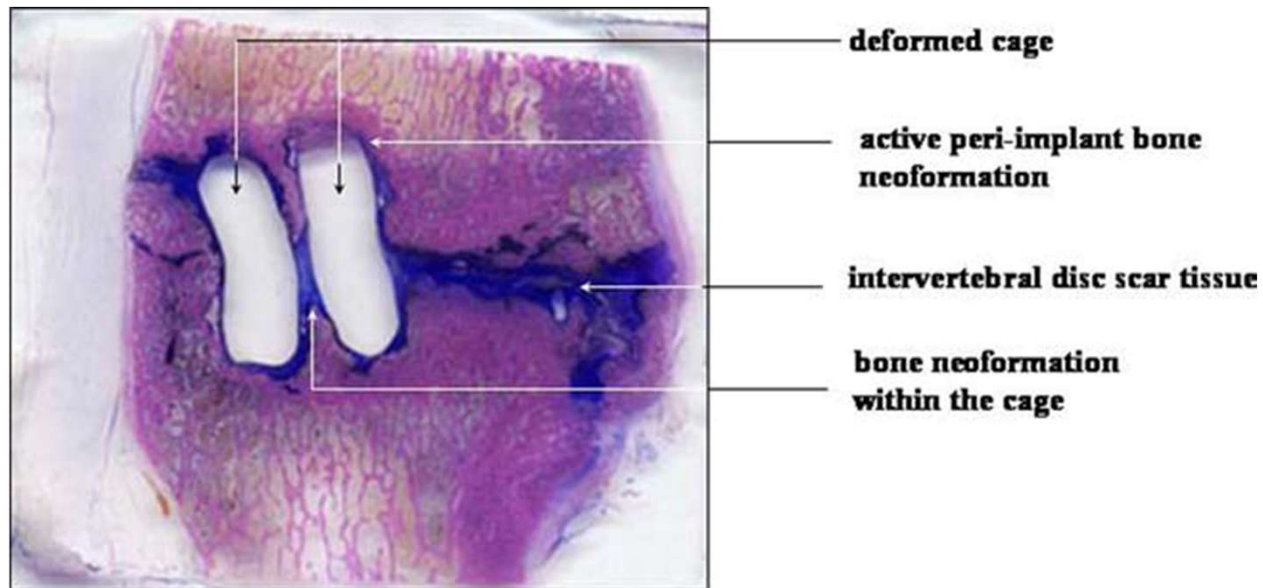


Figure 1 - Lazennec et al. 2006

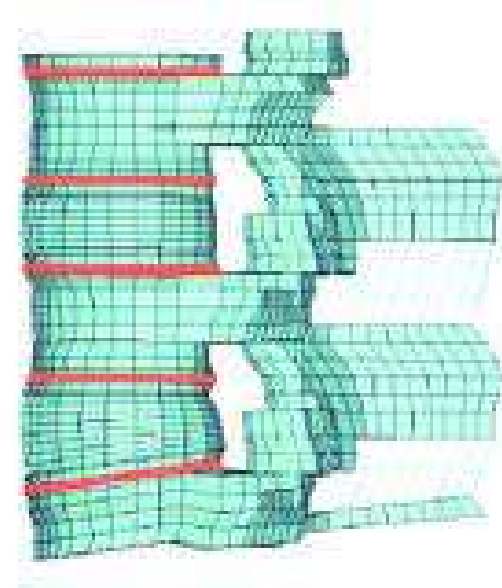
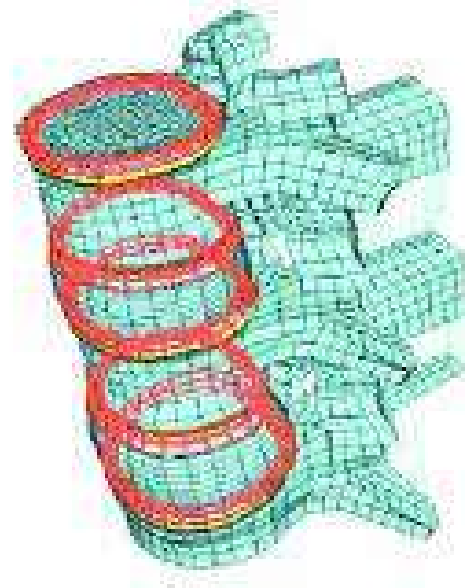


Figure 2 – Toth et al (2002)

1. Patient Safety (continued)

Apophyseal Ring

- Apophyseal Ring bears the most weight – very strong cortical bone.
- The centre of the vertebral body contains the softer, cancellous bone
- Some groups had the resorbable parts of the cage on the outside and the harder, permanent parts in the inside.
- It needs to be the other way around



The Best Bits....

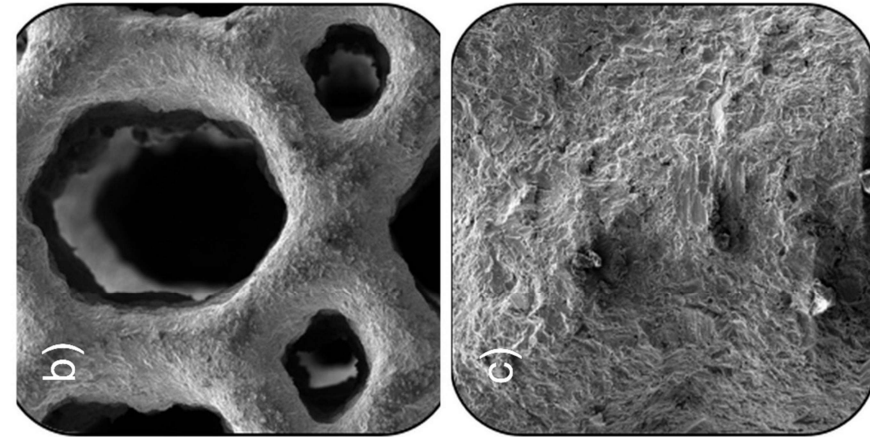
- ✓ The use of 2 materials to allow complementary aspects to overcome the weakness of just 1 material
 - Ti or Si_3N_4 or Coated PEEK as the permanent outer layer
 - Use of innovative resorbable materials for the inner (Mg Alloys)
- ✓ Use of i) porosity and ii) innovative lattice structures (e.g. TPMS lattices) to offset modulus issues with harder materials like Ti.
- ✓ Coating to slow Mg degradation (as likely too quick for new bone growth)
- ✓ Use of antimicrobial coatings (Silver)

Some 'builds' on your best bits

- Copper vs Silver for antimicrobial coating?
 - Copper mimics a hypoxic signal and leads to upregulated angiogenesis (blood vessels are important for bone formation)
- Slot-in design for a personalised resorbable inner?
 - Could have a fast, medium and slow resorbing inner, and the surgeon could assess the bone quality during surgery and select a more patient specific resorption profile vs one-size-fits-all resorption rate.

My design

- 3D printed Ti outer to sit on the Apophyseal Ring
- Open, porous structure with concave pores and submicron surface features
- A slot-in resorbable inner made, at least in part, from a Mg alloy to allow personalisation at point of use (fast, medium and slow)...likely to be coated (chitosan)
- In the very centre, a bone graft chamber for point of use addition of bone graft substitutes (e.g. rhBMP-2)



Summary

- Chronic low back pain
 - World Health Organisation – leading cause of disability worldwide
 - Debilitating, socially isolating
- Biomedical engineering has the potential to significantly improve outcomes for patients, and change lives.